

Health Trail Template Catalog: Implementation- Ready Research for Claude

TL;DR

- This catalog specifies **14 Situation Templates, 16 Project Templates, 16 Progress Presets, and 11 Standing Instruction Starters** for a local-first, record-only caregiver app, each as a structured block with contact roles, care threads, plain-language checklists, document slots, forward/folded sections, rights citations, and advice-risk flags.
- **Build order is driven by US caregiver population data:** skilled nursing/nursing home, hospital stay, and home care serve the largest

caregiver populations and ship first; pediatric serious illness, hospice, and dialysis are smaller but high-burden and ship later.

- **Hard design constraints**
throughout: templates are structure only (never advice), every string must translate cleanly into English, Spanish, Chinese, and Arabic (avoid the cognate traps for "hospice," "power of attorney," and "social worker"), and rights must be stated precisely (federal entitlement vs mere request).

Key Findings

Caregiver population and build-order basis

The NAC/AARP "Caregiving in the U.S. 2020" study counts **53 million** family

caregivers, more than one in five Americans, up from 43.5 million in 2015. The estimated prevalence of caring for an adult is **19.2 percent, or 47.9 million Americans, up from 16.6 percent in 2015.** On average, caregivers spend **23.7 hours a week** providing care, with one in three (32%) providing care for 21 hours or more. **More Americans (26%) are caring for someone with Alzheimer's disease or dementia, up from 22% in 2015;** 45% care for someone with two or more conditions, and 26% report difficulty coordinating care. Because dementia is the single largest condition category, memory care and home dementia caregiving are high-priority builds. Nursing home, hospital, and home settings dominate raw caregiver counts, so those situation templates ship first.

The rights vs requests distinction (critical for legal accuracy)

Federal law backs several family notification and participation rights in Medicare/Medicaid-certified nursing homes under the Nursing Home Reform Act (1987), codified at 42 CFR Part 483:

- **42 CFR 483.10(g)(14)** (formerly cited as 483.10(b)(11)): the facility must immediately inform the resident, consult the physician, and notify the resident's legal representative or interested family member of (A) an accident with injury that has potential for requiring physician intervention, (B) a significant change in physical/mental/psychosocial status, (C) a need to alter treatment

significantly, or (D) a decision to transfer or discharge.

- **42 CFR 483.10(b)(2):** resident/representative right to access records within 24 hours (excluding weekends/holidays), and to purchase copies at community-standard cost with 2 working days notice.
- **42 CFR 483.21:** comprehensive person-centered care planning; baseline care plan within 48 hours, comprehensive plan within 21 days, reviewed quarterly. Residents and representatives are partners with the right to attend and participate in care plan meetings.
- **42 CFR 483.15:** admission, transfer, and discharge rights, including bed-hold notice and involuntary-

discharge protections. Involuntary discharge is permitted only for six enumerated reasons under 42 USC 1395i-3(c)(2)(A)/1396r; at least 30 days written notice is generally required; resident may appeal and remain during appeal unless health/safety endangered.

These are genuine federal rights. By contrast, notification before a roommate change, or consent before every outside billable service, are best framed as **requests** the family makes, not guaranteed entitlements (though outside-service surprise billing does intersect with the federal No Surprises Act good-faith-estimate rules for uninsured/self-pay and out-of-network notice-and-consent, 45 CFR 149.610). The app must not label a request as a right.

CMS "Your Discharge Planning Checklist" structure (mirror, never copy)

The official CMS pamphlet (Product No. 11376) is organized as: action items with a notes column; a drug list table (drug name, what it does, dose, how to take, when to take); an appointments table; a "for a caregiver" subsection; and a fast-appeal rights box. Its action items include writing down names/phone numbers to call, asking about complications to watch for, building and reviewing the drug list, asking where care happens after discharge, asking about medical equipment, assessing help needed with daily activities, asking staff to demonstrate special-skill tasks, talking to a social worker about coping and insurance, and asking for written discharge instructions and a health-status summary. Health Trail

should mirror this structure (record fields and administrative prompts) but write its own strings and strip anything instructional.

Emergency Card fields (verified against File of Life / EMS)

File of Life and L.I.F.E. File programs (run by local fire/EMS) consistently list: patient name, date of birth, emergency contacts, health insurance/policy info, health conditions, medications with dosages, allergies, recent surgeries, physician name/number, religion, and whether a healthcare proxy/living will/DNR exists plus where the original lives. Programs note the card is kept somewhere responders know to look, and that mobility/communication/language/dementia notes help. Distinct from ICE (In Case

of Emergency) cards, which carry only contact numbers.

Cultural/linguistic design findings (design-relevant, sourced)

- **"Hospice" has no clean equivalent and carries stigma in all three non-English languages.** The Spanish cognate "hospicio" connotes an orphanage or poorhouse. In Chinese, common translations 临终关怀 ("last minute care") and 姑息治疗 (back-translates to "do-nothing care") are described by geriatrician Cynthia X. Pan, MD as "archaic and actually scary"; she proposes 安宁照顾 (comfort/peace care) and 舒缓治疗 (palliative). Arabic renderings split between دار العجزة (almshouse) and رعاية المحتضرين (care of the dying); الرعاية التلطيفية (palliative care) is

safest. Use descriptive phrasing plus a plain gloss, not the bare cognate.

- **Advance-directive terms are low-awareness or concept-mismatched.** Foundational work by Blackhall, Murphy, Frank, Michel, and Azen ("Ethnicity and attitudes toward patient autonomy," JAMA 1995;274(10):820-825, N=800, LA County) found that "Korean Americans (47%) and Mexican Americans (65%) were significantly less likely than European Americans (87%) and African Americans (88%) to believe that a patient should be told the diagnosis of metastatic cancer," reflecting a family-centered rather than individual-autonomy decision model. Confucian familism (Yang et al., IJERPH 2022) gives the

family, not the individual, decision priority. Arab family decision-making is often collective and male-led (eldest son, uncle, grandfather), with documented nondisclosure norms. A Shanghai study found four distinct decision-style profiles in one population, so the app should ask, not assume. (PubMed)

- **Design takeaways:** support multiple decision-makers / a designated family "point person"; provide a "who decides / who receives information" setting; default Spanish copy to formal *usted* and address elders as Señor/Señora; and avoid literal "social worker" labels (the term maps poorly in Chinese and carries child-protective-services stigma in

immigrant communities). Prefer function-descriptive role labels.

Details

TEMPLATE TYPE 1: SITUATION TEMPLATES

Each block: name; target setting; ~6 contact roles; 4-5 care threads; ~10 administrative checklist items; document slots; forward vs folded sections; distinctive organizational burden; rights citations; advice-risk flags. All checklist items are administrative/organizational, never clinical.

1.1 Skilled Nursing Facility / Nursing Home (long-term custodial)

- Contact roles: Director of Nursing (DON), Charge Nurse, Social Worker,

MDS Coordinator, Administrator, Business Office/Billing

- Care threads: Nursing, Personal care/CNA, Activities, Dietary, Social services
- Checklist: Get direct phone numbers for the unit and DON; get the current medication list; confirm who calls the family and when; write down room and bed details; meet the assigned social worker; locate the written grievance procedure and state survey agency contact; ask the care plan meeting schedule; confirm visiting rules; get the billing contact and what is/isn't covered; note the ombudsman contact
- Document slots: Admission agreement, resident rights notice, current med list, care plan summary,

insurance/Medicaid card, advance directive/POLST

- Forward: Standing Instructions, Trail, Care Threads, Money. Folded: Appointments, Progress
- Distinctive burden: tracking care plan meetings and MDS assessment cycles; watching for involuntary-discharge notices; grievance and ombudsman routes; long-run cost/Medicaid interaction
- Rights: care plan participation (483.21); notification of changes (483.10(g)(14)); records access (483.10(b)(2)); discharge protections (483.15)
- Advice-risk flags: keep checklist administrative; never prompt families to assess medical status

1.2 Short-Term Rehab (SNF rehab stay post-hospital)

- Contact roles: Case Manager/Discharge Planner, Charge Nurse, PT lead, OT lead, Social Worker, Business Office
- Care threads: Physical therapy, Occupational therapy, Speech therapy, Nursing, Discharge planning
- Checklist: Get the expected length-of-stay estimate in writing; get therapy schedule; confirm who to call about progress; ask for the projected discharge date and destination; identify the case manager; ask how Medicare days are tracked; request notice rules for coverage ending; confirm home equipment plans; get

the billing contact; note the fast-appeal contact (BFCC-QIO)

- Document slots: Hospital discharge summary, therapy schedule, Notice of Medicare Non-Coverage, med list, insurance card
- Forward: Progress, Appointments, Trail, Projects. Folded: Money (unless dispute), Standing Instructions
- Distinctive burden: tracking the Medicare 100-day benefit clock, watching for the Notice of Medicare Non-Coverage, preparing a fast appeal; coordinating discharge home
- Rights: fast appeal of service termination via BFCC-QIO; Jimmo standard (coverage requires need for skilled care, not "improvement")
- Advice-risk flags: record clinician-stated milestones only; do not

interpret therapy progress

1.3 Inpatient Rehabilitation Facility (IRF)

- Contact roles: Case Manager, Rehab Physician (Physiatrist), Charge Nurse, PT/OT/Speech leads, Social Worker
- Care threads: Physical therapy, Occupational therapy, Speech therapy, Rehab nursing, Discharge planning
- Checklist: Confirm the therapy-hours expectation; get the team conference schedule; identify the case manager and social worker; get direct unit phone; ask who updates the family and when; request the projected discharge date/destination; ask about equipment and home modifications; confirm insurance

authorization status; get the billing contact; note appeal contacts

- Document slots: Hospital transfer records, rehab plan, therapy schedule, insurance authorization letters, med list
- Forward: Progress, Appointments, Projects, Trail. Folded: Money (unless dispute)
- Distinctive burden: intensive daily therapy and weekly team conferences; families track team-conference dates and authorization renewals
- Advice-risk flags: milestones recorded as clinician-stated only

1.4 Hospital Stay (planned and unplanned/ER admission)

- Contact roles: Hospitalist, Charge Nurse, Case Manager/Discharge Planner, Social Worker, Patient Advocate/Patient Relations, Attending/Specialist
- Care threads: Nursing, Physician/hospitalist care, Discharge planning, Consulting specialists, (optional) therapy
- Checklist: Write down the hospitalist and nurse names each shift; get the unit direct phone; ask about the plan for the day; confirm inpatient vs observation status; ask for the discharge planning contact; note the "Important Message from Medicare" and appeal rights; build/verify the med list; ask what to expect at discharge; identify the patient

advocate; record

insurance/authorization contact

- Document slots: Important Message from Medicare, Medicare Outpatient Observation Notice (MOON) if applicable, discharge summary, med list, test result copies (with where original lives), insurance card
- Forward: Trail, Appointments, Documents, Standing Instructions. Folded: Money (until bills), Progress
- Distinctive burden: fast-moving shift changes and rotating staff names; the inpatient-vs-observation status question (affects later SNF coverage); short discharge windows; fast-appeal deadline
- Rights: fast discharge appeal via BFCC-QIO; Important Message from

Medicare within 2 days of admission;
MOON if in observation 24+ hours

- Advice-risk flags: record only what staff say; no symptom interpretation

1.5 Home Care with Family Caregiving (no agency)

- Contact roles: Primary care physician, Specialist(s), Pharmacy, Preferred hospital/ER, Backup family contact, Durable medical equipment supplier
- Care threads: Daily personal care, Medication management (record only), Appointments/transport, Household/errands, Safety
- Checklist: Build the contact card of doctors and pharmacy; write down the preferred hospital; assemble the med list; note insurance and ID card

locations; set up the document folder; record allergy and blood type info for the Emergency Card; note advance-directive location; list equipment and suppliers; write down who to call for what; set up a shared cost sheet

- Document slots: Med list, insurance cards, advance directive/POLST, ID documents, equipment receipts/warranties
- Forward: Emergency Card, Trail, Appointments, Medications. Folded: Projects (until needed)
- Distinctive burden: the family is the coordinator with no institutional staff; everything rests on them; the Emergency Card matters most here
- Advice-risk flags: Medications section is record-only; no reminders or dosing guidance

1.6 Home Health (agency-provided)

- Contact roles: Agency Scheduler, Home Health Nurse (case manager RN), Home Health Aide, PT/OT (if ordered), Agency intake/billing, Ordering physician
- Care threads: Skilled nursing visits, Physical/occupational therapy, Home health aide, Wound care (if applicable), Coordination with physician
- Checklist: Get the agency's main and after-hours numbers; get the assigned nurse's name and schedule; confirm visit frequency; ask what each visit covers; confirm who to call for a missed visit; get the ordering physician's contact; ask how supplies are provided; note Medicare coverage terms and the non-coverage notice

rule; confirm the plan-of-care review date; record billing contact

- Document slots: Plan of care/physician orders, visit schedule, Notice of Medicare Non-Coverage, med list, insurance card
- Forward: Appointments, Care Threads, Trail, Standing Instructions. Folded: Money (unless dispute)
- Distinctive burden: tracking scheduled visits and no-shows across multiple staff; the 60-day certification episode and coverage-ending notices; supply logistics
- Rights: fast appeal of home health termination via BFCC-QIO (Notice of Medicare Non-Coverage on the second-to-last visit)
- Advice-risk flags: record visit occurrence and content as stated; no

clinical assessment

1.7 Assisted Living

- Contact roles: Executive Director, Wellness/Health Services Director (nurse), Care/Med Aide, Concierge/Front Desk, Business Office, Activities Director
- Care threads: Personal care/assistance, Medication assistance (record only), Dining, Activities, Housekeeping/laundry
- Checklist: Get the community's main and after-hours numbers; identify the wellness director; get the service plan and what the monthly fee covers; ask about add-on service charges; confirm who calls the family and when; note the move-in agreement terms; ask about level-of-

care reassessment triggers; confirm dining and dietary handling; record billing contact; note grievance process

- Document slots: Residency agreement, service/care plan, fee schedule, med list, insurance/ID, advance directive
- Forward: Money, Trail, Standing Instructions, Appointments. Folded: Progress
- Distinctive burden: assisted living is largely state-regulated (not federal), so rights vary; the key burden is tracking level-of-care reassessments that raise fees and watching add-on charges; move-out risk when needs exceed the community's license
- State variance: HIGH (licensed and regulated state-by-state)

- Advice-risk flags: none if administrative

1.8 Memory Care / Dementia Unit

- Contact roles: Wellness/Health Services Director, Memory Care Director, Care Aide, Social Worker/Family Liaison, Business Office, Activities/Engagement lead
- Care threads: Personal care, Medication assistance (record only), Behavioral/engagement observations, Dining/nutrition support, Safety/wandering precautions
- Checklist: Get unit direct and after-hours numbers; identify the memory care director and liaison; get the care plan and reassessment schedule; confirm who calls the family and

when; note the visiting and secured-unit access rules; record dietary and food preferences on file; note the elopement/safety notification rule; confirm how belongings are labeled; record billing contact; note grievance/ombudsman route

- Document slots: Care plan, residency/admission agreement, med list, advance directive/POLST, ID, personal preferences/life-history sheet
- Forward: Standing Instructions, Trail, Progress (behavior observations), Emergency Card. Folded: Money (unless dispute)
- Distinctive burden: tracking behavior and mood observations over time; safety/elopement notifications; the

family often becomes the legal decision-maker as capacity declines

- Cultural note: use observation-only language in Progress; support a designated family point person and multiple decision-makers
- Advice-risk flags: HIGH.
Mood/behavior tracking must stay observational ("what I saw," "what staff reported"), never diagnostic

1.9 Hospice (in-home and facility)

- Contact roles: Hospice Nurse (case manager RN), Hospice Aide, Social Worker, Chaplain, On-call/after-hours nurse line, Medical Director/physician
- Care threads: Comfort-focused nursing, Personal care/aide,

Emotional/social support, Spiritual support, Bereavement support

- Checklist: Get the 24/7 on-call number; identify the assigned nurse and visit schedule; confirm who to call after hours; note the aide and chaplain visit cadence; record what equipment/supplies hospice provides; confirm the medication delivery arrangement; note who to call at time of death; record the DNR/POLST location; confirm the plan-of-care review; note the bereavement contact
- Document slots: Hospice election/consent, plan of care, DNR/POLST, med list, equipment list, important-contacts sheet
- Forward: Emergency Card, Standing Instructions, Trail, Care Threads.

Folded: Money, Projects

- Distinctive burden: coordinating a 24/7 on-call relationship and end-of-life communication protocol; managing the DNR/POLST so EMS follow it
- Cultural note: LABEL WITH CARE. Do not use bare cognates ("hospicio"; 临终关怀; دار العجزة). Prefer descriptive comfort-care phrasing plus a plain gloss in each language
- Advice-risk flags: HIGH. No guidance on symptoms, medications, or what to expect medically; record only

1.10 Outpatient Dialysis

- Contact roles: Charge Nurse/Nurse Manager, Dialysis Charge Tech/PCT, Social Worker, Renal Dietitian,

Nephrologist, Transportation coordinator

- Care threads: Dialysis sessions, Vascular access care (record only), Nutrition counseling, Transportation, Nephrology follow-up
- Checklist: Get the clinic's main and scheduling numbers; confirm the session schedule (days/shift); identify the charge nurse and social worker; note the transportation arrangement; confirm who to call about a missed session; record the nephrologist contact; ask how lab results are shared; note the clinic's holiday schedule; record billing/insurance contact; note the grievance process

- Document slots: Treatment schedule, transportation plan, med list, insurance/Medicare card, advance directive
- Forward: Appointments, Progress (session log), Trail, Care Threads. Folded: Projects
- Distinctive burden: the relentless 3x/week schedule and transportation logistics; tracking sessions attended/missed and how the person felt after
- Advice-risk flags: session log records attendance/duration/self-reported feeling only; no clinical interpretation

1.11 Outpatient Cancer Treatment / Infusion

- Contact roles: Oncology Nurse Navigator, Infusion Charge Nurse,

Oncologist, Scheduler, Financial counselor/Social Worker, Specialty Pharmacy

- Care threads: Chemo/infusion cycles, Symptom/side-effect observation (record only), Appointments/scans, Nutrition/support services, Financial counseling
- Checklist: Get the navigator's and clinic's numbers; get the treatment cycle calendar; confirm who to call about side effects after hours; identify the oncologist and infusion contacts; note the scan/lab schedule; record the pharmacy and prior-auth status; confirm the financial counselor contact; note transportation plans; record insurance contact; ask how results are shared

- Document slots: Treatment plan/cycle calendar, consent forms, med list, prior-authorization letters, insurance card, advance directive
- Forward: Appointments, Progress (cycle log), Trail, Money. Folded: Standing Instructions
- Distinctive burden: tracking multi-week cycles, scan dates, prior authorizations, and side-effect observations; heavy financial/prior-auth coordination
- Advice-risk flags: side-effect log is observational; no symptom management advice

1.12 Pediatric Serious Illness (child in ongoing treatment)

- Contact roles: Pediatric Care Coordinator/Nurse Navigator,

Attending Pediatric Specialist, Child Life Specialist, Social Worker, School Liaison, Scheduler

- Care threads: Primary treatment (e.g., oncology/specialty), Therapies (PT/OT/speech as applicable), School/education continuity, Family/psychosocial support, Appointments/procedures
- Checklist: Get the care coordinator's number; build the specialist contact card; identify the child life specialist and social worker; get the treatment/appointment calendar; confirm who to call after hours; note the school liaison and education plan (504/IEP) contact; record insurance/prior-auth status; note the pharmacy; confirm the procedure-

prep contact; record billing/financial counselor

- Document slots: Treatment plan, appointment calendar, school/education plan, consent forms, med list, insurance and prior-auth letters, growth records
- Forward: Appointments, Care Threads, Progress (growth + treatment), Trail. Folded: Money (until bills)
- Distinctive burden: juggling clinical treatment with school continuity and normal development; both parents often co-manage (multiple point persons); child life coordination is unique to pediatrics
- Advice-risk flags: growth tracking records values only; no assessment

1.13 Post-Stroke Recovery (across settings)

- Contact roles: Case Manager, Rehab Physician/Neurologist, PT/OT/Speech leads, Charge Nurse, Social Worker, Primary care physician
- Care threads: Physical therapy, Occupational therapy, Speech/language therapy, Nursing/medication (record only), Discharge/transition planning
- Checklist: Get the case manager's number; identify the therapy leads; get the therapy schedule; confirm who updates the family and when; note the projected setting transitions (hospital to rehab to home); record equipment and home-modification plans; confirm follow-up appointments; note insurance

authorization status; record the primary care contact; note appeal contacts

- Document slots: Discharge/transfer summaries, therapy schedules, equipment list, med list, insurance authorizations, advance directive
- Forward: Progress (mobility + speech milestones), Care Threads, Appointments, Projects. Folded: Money (unless dispute)
- Distinctive burden: recovery crosses multiple settings over months, so the family tracks parallel PT/OT/speech threads and repeated transitions; milestone-heavy
- Advice-risk flags: mobility/speech progress recorded as clinician-stated milestones; no interpretation

1.14 Home Dementia Caregiving (family at home, no facility)

- Contact roles: Primary care physician, Neurologist/memory specialist, Pharmacy, Preferred hospital/ER, Backup family/decision-maker, Adult day program/respite contact
- Care threads: Daily personal care, Medication management (record only), Behavior/mood observations, Safety/wandering precautions, Respite/support
- Checklist: Build the doctor and pharmacy contact card; write down the preferred hospital; assemble the med list; complete the Emergency Card (allergies, blood type, DNR/POA status); note the advance-directive location; record safety notes

(wandering, locks); list respite/day-program contacts; note who the backup decision-maker is; set up the document folder; record the cost sheet

- Document slots: Med list, advance directive/POA, insurance/ID, safety/preferences sheet, respite program info
- Forward: Emergency Card, Standing Instructions, Progress (behavior), Trail. Folded: Money
- Distinctive burden: 24/7 supervision with no staff; safety and wandering; caregiver burnout and respite planning; the family holds legal decision-making
- Advice-risk flags: HIGH. Behavior observations only; no clinical assessment or management advice

TEMPLATE TYPE 2: PROJECT TEMPLATES

Each block: name; ~10-15 plain-language administrative steps; contact role types; documents; timeline; failure points; federal/uniform vs state-variant flag; rights citations; advice-risk flags.

2.1 Medicaid Long-Term-Care Application

- Steps: Confirm which Medicaid program applies; find the local Medicaid office contact; get the application form; gather ID and residency proof; gather income documents; gather asset/bank statements (often 5 years); document any spend-down; submit the application; get a confirmation/case number; identify the assigned

caseworker; respond to requests for more documents; track the decision deadline; if denied, note the appeal deadline; keep copies

- Contact roles: Medicaid caseworker, Elder law attorney (optional), Facility business office, Bank
- Documents: Application, ID, income proof, bank statements, property records, insurance policies, spend-down receipts
- Timeline: often 45-90 days; longer if documents missing
- Failure points: missing bank statements; unexplained transfers within the lookback; missed document-request deadlines
- State variance: HIGH (income/asset limits, lookback, forms vary)

- Advice-risk flags: record steps only; no financial/eligibility advice

2.2 Medicaid HCBS Community Waiver Application (with waitlist)

- Steps: Identify the state's HCBS waiver(s); confirm the waiver fits the need; check functional (level-of-care) eligibility; check financial eligibility; gather documents; find the local Medicaid or Area Agency on Aging contact; submit the application; ask whether there is a waitlist; get the waitlist placement date and any priority status; get a case number; track "Medicaid pending" status; respond to document requests; note the reassessment schedule; watch for the invitation-to-enroll letter

- Contact roles: Medicaid/waiver caseworker, Area Agency on Aging, Managed care entity (if applicable), Elder law attorney (optional)
- Documents: Application, level-of-care assessment, income/asset proof, waitlist placement letter, invitation letter
- Timeline: application 4-8 weeks to several months; waitlist can be months to years in high-demand states
- Failure points: waitlist limbo; discovering ineligibility only at the top of the list (in states that don't screen first); missed reassessment
- State variance: HIGH. Per KFF's report *A Look at Waiting Lists for Medicaid Home- and Community-Based Services from 2016 to 2025*

(2025), the number of states with waiting lists currently sits at 41, with over 600,000 people on lists in 2025; most states screen for eligibility before placement, but six (Florida, Iowa, Oklahoma, Oregon, South Carolina, Texas) place applicants without screening

- Advice-risk flags: record only

2.3 Medicare Coverage Denial Appeal (including SNF/Jimmo and expedited discharge)

- Steps: Read the denial or non-coverage notice carefully; note which notice it is (Important Message from Medicare, Notice of Medicare Non-Coverage, or a claim denial); identify the deadline; for a discharge/service-ending case, call the BFCC-QIO by

the stated deadline (midnight of discharge day for hospital); request the Detailed Notice; gather supporting records or a doctor's letter; note the Jimmo point (coverage does not require "improvement," only need for skilled care); track the QIO decision; if denied, note the next appeal level (QIC reconsideration); keep copies; record who you spoke with and when

- Contact roles: BFCC-QIO, Facility case manager, Treating physician, SHIP counselor
- Documents: The notice received, Detailed Notice of Discharge/Non-Coverage, supporting records, physician letter, appeal confirmations

- Timeline: The BFCC-QIO must decide within one calendar day after receiving needed information (per Medicare.gov "Fast appeals"); the second-level QIC reconsideration must decide within 72 hours (Center for Medicare Advocacy)
- Failure points: missing the midnight/noon deadline; not requesting the Detailed Notice; assuming "no improvement" ends coverage (Jimmo says otherwise)
- Federal/uniform: YES. The BFCC-QIO function is split: Acentra Health (formerly Kepro) serves 29 states (CMS Regions 1, 4, 6, 8, 10) and Commence Health (formerly Livanta, renamed Aug. 18, 2025) serves the rest (Regions 2, 3, 5, 7, 9)

- Rights: fast appeal via BFCC-QIO; Jimmo v. Sebelius (2013) maintenance-coverage standard
- Advice-risk flags: state the Jimmo standard as a factual record of the rule, not medical advice about the person's need

2.4 Private Insurance Prior Authorization Appeal

- Steps: Get the denial letter; find the reason; note the appeal deadline; call the insurer to confirm the process; request the plan's clinical criteria; ask the ordering provider for a letter of medical necessity; gather supporting records; submit the internal appeal; get a reference number; track the decision; if denied, note the external

review option; keep copies; record all calls

- Contact roles: Insurer member services, Insurer appeals department, Ordering provider's office, Employer benefits (if employer plan), State insurance department (external review)
- Documents: Denial letter, plan clinical criteria, letter of medical necessity, supporting records, appeal submissions
- Timeline: internal appeals often 30-60 days (expedited faster); external review follows internal
- Failure points: missing the appeal deadline; not getting the medical-necessity letter; not escalating to external review

- State variance: MODERATE
(external review governed by state law for state-regulated plans; ERISA/self-funded plans differ)
- Advice-risk flags: record only; no advice on medical necessity

2.5 Hospital Discharge Planning (mirrors CMS checklist structure)

- Steps: Write down who to call with questions; ask where care happens after discharge; note options offered; ask about equipment needs and who arranges delivery; identify help needed with daily activities; ask staff to demonstrate any special-skill tasks; talk to the social worker about coping and cost; confirm insurance coverage questions; get written discharge instructions and a health-

status summary; build the updated
med list; record follow-up
appointments; note the fast-appeal
contact if discharge feels too soon

- Contact roles: Discharge planner/case manager, Social worker, Hospitalist, Home health/DME coordinator, BFCC-QIO (if appealing)
- Documents: Written discharge instructions, health-status summary, med list, equipment orders, follow-up appointment list
- Timeline: compressed, often 1-3 days notice
- Failure points: discharge before equipment/home care is arranged; unclear follow-up; missed appeal window
- Federal/uniform: YES (CMS discharge planning Conditions of

Participation)

- Advice-risk flags: mirror structure only; strip clinical instruction; record what staff provide

2.6 Medical Records Request (HIPAA right of access)

- Steps: Identify who holds the records; find their records-request form or process; confirm you are the patient or authorized personal representative; submit a written request specifying the format wanted (paper or electronic); note the date submitted; note the 30-day deadline (one 30-day extension allowed with written notice); confirm the fee is cost-based only; follow up if late; if stonewalled, note the OCR complaint

option; keep copies of the request and responses

- Contact roles: Provider/hospital health information management (HIM), Plan member services, HHS Office for Civil Rights (if complaint)
- Documents: The written request, proof of representative status, records received, fee receipts, any denial letter
- Timeline: 30 calendar days, one 30-day extension permitted (off-site records may extend timelines)
- Failure points: being told to use only a portal; excessive fees; delay past 30 days; being asked to state a reason (not required)
- Federal/uniform: YES (45 CFR 164.524; HIPAA is a federal floor, stricter state law can apply)

- Rights: cost-based fees only (labor for copying, supplies, postage); no charge to inspect; since 2019 OCR has fined entities for delay or overcharging
- Advice-risk flags: none

2.7 Long-Term-Care Ombudsman / State Survey Agency Complaint

- Steps: Write down what happened with dates and names; decide the route (ombudsman for advocacy/mediation, state survey agency for enforcement); find the ombudsman contact; find the state survey agency contact; gather any documentation or photos; file the complaint (can be anonymous); note the reference number; note the priority level assigned; track the

investigation timeline; keep in contact and ask for updates; record the outcome and any plan of correction

- Contact roles: Long-Term Care Ombudsman, State Survey Agency, Adult Protective Services (if abuse), CMS regional office
- Documents: Complaint text, incident notes, photos, medical records, correspondence
- Timeline: immediate-jeopardy investigations begin within 2 working days; high-priority within 10 working days; facility submits plan of correction within 10 calendar days of deficiencies
- Failure points: low-priority complaints deferred to the next

survey; losing the reference number;
not following up

- Federal/uniform: framework is federal (Older Americans Act ombudsman program; CMS survey process); state agencies administer
- Advice-risk flags: none

2.8 Social Security Disability (SSDI/SSI) Application

- Steps: Check eligibility (work history for SSDI; income/assets for SSI); download the Adult Disability Starter Kit; gather personal ID (birth certificate, SSN); list all doctors, hospitals, and treatment dates; list work history (15 years); gather medications list; complete the application (online for SSDI; form/interview for SSI); submit; get a

confirmation/claim number; respond to SSA requests; track the decision; if denied, note the reconsideration/appeal deadline; keep copies

- Contact roles: SSA field office, Disability advocate/attorney (optional), Treating physicians (records), Two non-medical contacts who know the condition
- Documents: ID, medical provider list, work history, med list, application confirmation, denial/appeal letters
- Timeline: initial decision often 3-6 months; many initial claims denied; appeals add months
- Failure points: missing medical documentation; missed appeal deadlines; incomplete work history

- Federal/uniform: YES (SSA is federal); SSI resource limits are federal (\$2,000 individual / \$3,000 couple for 2025)
- Advice-risk flags: record only; no advice on disability qualification

2.9 VA Aid and Attendance / VA Caregiver Support (PCAFC)

- Steps: Determine which program (Aid & Attendance pension add-on vs PCAFC clinical caregiver program); for PCAFC confirm the veteran's eligibility (service-connected disability rating 70%+, need for 6+ months of personal care); download VA Form 10-10CG (PCAFC) or the pension/A&A forms; complete the joint application (veteran + caregiver for PCAFC);

submit online or by mail; contact the Caregiver Support Line; track the application through the review steps; complete the home-care assessment; get the written determination; if denied, note the review/appeal option; keep copies

- Contact roles: VA Caregiver Support Program team, VA Caregiver Support Line (1-855-260-3274), VSO (Veterans Service Officer), Veteran's VA physician
- Documents: VA Form 10-10CG (PCAFC), discharge/service records, disability rating letter, medical evidence, determination letter
- Timeline: multi-step review; PCAFC has a four-stage review/appeal process

- Failure points: A&A family-caregiver arrangement documented incorrectly (common denial cause); incomplete joint application; missing rating evidence
- Federal/uniform: YES (VA is federal)
- Advice-risk flags: none

2.10 Guardianship / Conservatorship Initiation (high level)

- Steps: Understand that this is a court process that varies by state; consider whether less-restrictive options exist (POA, supported decision-making) first; find the local probate/court self-help resource; identify the required petition forms; gather medical/capacity documentation; identify who must be notified; file the petition; note the hearing date;

attend the hearing; get the court order; understand ongoing reporting duties; keep copies

- Contact roles: Probate court clerk, Attorney (usually needed), Court-appointed evaluator/physician, Court investigator/guardian ad litem
- Documents: Petition, capacity/medical evidence, court order, ongoing accounting/reports
- Timeline: weeks to months depending on court and contested status
- Failure points: choosing guardianship when a POA would suffice; missing reporting deadlines after appointment
- State variance: VERY HIGH (guardianship law is entirely state-

specific; keep template generic with a "your state may differ" posture)

- Advice-risk flags: not legal advice; record steps only

2.11 Power of Attorney and Advance Directive Setup

- Steps: Learn the distinctions (financial POA; healthcare POA/healthcare proxy; living will; POLST/MOLST for the seriously ill); decide who the agent(s) will be; get the state's forms; complete the forms with the person while they have capacity; sign with required witnesses/notary; distribute copies to agents, doctors, and the facility; note where originals live; add DNR/POLST if applicable and physician-signed;

record all locations on the
Emergency Card; review periodically

- Contact roles: Attorney (optional),
Notary, Primary physician (for
POLST/MOLST), Named agent(s)
- Documents: Financial POA,
healthcare POA/proxy, living will,
POLST/MOLST (if applicable), DNR
- Timeline: can be same-week; POLST
requires a physician
- Failure points: forms not
witnessed/notarized correctly;
agents/doctors don't have copies;
confusing a living will (planning)
with a POLST (medical order EMS
can follow)
- State variance: HIGH (forms, terms,
and POLST availability vary;
"healthcare proxy" vs "healthcare
POA" is a naming difference by state)

- Cultural note: gloss POA/advance directive plainly in all four languages; awareness is low and terms mismatch; support multiple decision-makers and a family point person
- Advice-risk flags: not legal advice; the app records the setup and locations, does not draft or advise

2.12 FMLA Leave Request (for the caregiver)

- Steps: Check if your employer is covered and you are eligible; note you don't have to say "FMLA" first, just give enough info; give notice (30 days if foreseeable, as soon as practicable if not); ask HR for the paperwork; get the medical certification form (WH-380-F) to the family member's provider; return certification within

15 days; keep the eligibility notice (WH-381) and designation notice (WH-382); track intermittent leave hours used; keep copies of all notices

- Contact roles: Employer HR, Family member's healthcare provider, DOL Wage and Hour Division (if a problem)
- Documents: Notice to employer, WH-380-F certification, WH-381 eligibility notice, WH-382 designation notice, leave-tracking log
- Timeline: employer eligibility notice within 5 business days; certification due within 15 days; up to 12 workweeks unpaid leave (26 for military caregiver leave)
- Failure points: late certification; not tracking intermittent hours;

assuming leave is paid (it is unpaid unless combined with paid leave)

- Federal/uniform: YES (FMLA is federal; some states have stronger leave laws)
- Advice-risk flags: none

2.13 Facility-to-Facility Transfer Request

- Steps: Identify the reason and desired destination; confirm the receiving facility has a bed and accepts the payer; ask the current facility's case manager to initiate; confirm records and med list will transfer; confirm transport arrangements; note bed-hold/return rights at the current facility; get the transfer date; confirm insurance/Medicaid authorization; update the contact cards; note the

new room/bed details; keep copies of transfer paperwork

- Contact roles: Current facility case manager/social worker, Receiving facility admissions, Insurer/Medicaid, Transport service
- Documents: Transfer summary, med list, insurance authorization, bed-hold notice
- Timeline: days to weeks depending on bed availability and authorization
- Failure points: records not transferring; losing the bed at the origin; authorization gaps
- State variance: MODERATE (bed-hold rules vary; federal 483.15 sets baseline notice)
- Advice-risk flags: none

2.14 Appealing a Nursing Home Discharge/Eviction (involuntary discharge)

- Steps: Read the discharge notice; confirm it lists a reason, destination, and appeal rights; check the reason against the six federally allowed reasons; note the 30-day notice requirement; contact the ombudsman immediately; file the appeal with the state agency by the proposed discharge date; note that the resident can usually stay during the appeal; gather evidence (payment records, care records); prepare for the hearing; note bed-hold/readmission rights after hospitalization; keep copies
- Contact roles: Long-Term Care Ombudsman, State

Medicaid/hearings agency, Facility administrator, Attorney/legal aid

- Documents: Discharge notice, appeal request, payment records, care records, hearing materials
- Timeline: appeal generally must be filed by the discharge date; 30-day notice standard
- Failure points: "24-hour notices" (almost always unlawful); confusing Medicare coverage ending with an eviction; hospital "dumping" (refusing readmission)
- Federal/uniform: YES (Nursing Home Reform Act 1987; 42 USC 1395i-3(c)(2)/1396r; 42 CFR 483.15); state administers hearings
- Rights: only six allowed reasons; 30-day notice; right to appeal and

remain during appeal; right to return after hospitalization

- Advice-risk flags: not legal advice; record steps and rights as factual

2.15 Spend-Down / Asset Documentation for Medicaid Eligibility

- Steps: Understand the concept (reducing countable assets/income to the state limit); gather bank and asset statements (often 5 years back for the lookback); list all accounts and property; document any transfers or gifts; keep receipts for allowed spend-down (medical bills, home modifications, prepaid funeral); organize by date; identify what counts vs is exempt in your state; consider consulting an elder law attorney or Medicaid planner; submit

documentation with the application;
keep copies of everything

- Contact roles: Medicaid caseworker, Elder law attorney/Certified Medicaid Planner, Bank, Facility business office
- Documents: 5 years of bank statements, asset/property records, transfer documentation, spend-down receipts, exemption proof
- Timeline: runs parallel to the LTC Medicaid application; lookback commonly 5 years
- Failure points: undocumented transfers triggering a penalty period; missing statements; assuming assets are exempt when they aren't
- State variance: HIGH (limits, exemptions, lookback administration vary)

- Advice-risk flags: NOT financial/legal advice; record documents and steps only

2.16 Private Insurance / Medicare EOB and Bill Reconciliation

- Steps: Collect all bills and Explanation of Benefits (EOB) statements; match each bill to its EOB; note what insurance paid and what is owed; flag any charge that doesn't match; note billing/coding questions; call the provider billing office; call the insurer if the EOB is wrong; open a dispute if needed and get a reference number; track the dispute state; note payment deadlines; keep copies
- Contact roles: Provider billing office, Insurer member services, SHIP

counselor, State insurance
department (if unresolved)

- Documents: Bills, EOBs, dispute correspondence, payment records
 - Timeline: ongoing; disputes can take weeks
 - Failure points: paying a bill before the EOB arrives; balance billing; missing dispute windows
 - Advice-risk flags: none
-

TEMPLATE TYPE 3: PROGRESS PRESETS

Each: metric name; unit; entry cadence;
medication-start markers relevant?;
milestones vs continuous?; gap tolerance;
advice-risk flag.

3.1 Weight — unit: lb or kg; cadence:
weekly or as clinician asks; med-start

markers: YES (record medication start date as a marker for GLP-1/Ozempic-type context, not advice); milestones: no, continuous; gap tolerance: high; flag: record numbers only, no target/interpretation.

3.2 Blood Pressure — unit: mmHg (systolic/diastolic); cadence: as clinician asks, often daily; med-start markers: YES; milestones: no; gap tolerance: moderate; flag: never flag "high/low" or advise; record the reading and who took it.

3.3 Blood Glucose — unit: mg/dL; cadence: as clinician asks; med-start markers: YES (insulin/med start dates); milestones: no; gap tolerance: low if daily monitoring; flag: no interpretation, no dosing guidance.

3.4 Mobility / Rehab — unit: plain family language (distance walked in feet/steps;

assistance level as "no help / some help / a lot of help / could not"); cadence: per session or weekly; med-start markers: no; milestones: YES, milestone-heavy (first steps, first stairs, walker to cane); gap tolerance: high; flag: use plain language not clinical scales; record clinician-stated levels, don't assess.

3.5 Wound / Pressure Sore — unit: photo + date + stage-as-recorded-by-clinician; cadence: per dressing change or clinician visit; med-start markers: no; milestones: mixed; gap tolerance: low; flag: HIGH.

Family records the photo and the stage the clinician stated; never assigns a stage or assesses healing.

3.6 Pain Level — unit: 0-10 self-report; cadence: as needed or routine; med-start markers: YES; milestones: no; gap

tolerance: high; flag: record the person's self-report only; no management advice.

3.7 Sleep — unit: hours and/or plain notes ("woke 3 times"); cadence: daily; med-start markers: YES; milestones: no; gap tolerance: high; flag: observational only.

3.8 Appetite / Meals Eaten — unit: plain ("ate most / half / little / none"); cadence: per meal or daily; med-start markers: optional; milestones: no; gap tolerance: high; flag: observational; no nutrition advice.

3.9 Mood / Behavior Observations (dementia contexts) — unit: short observation notes + optional plain tags (calm/agitated/withdrawn); cadence: daily or per event; med-start markers: YES; milestones: no; gap tolerance: high; flag: HIGH. Observation-only language ("what I

saw," "what staff reported"); never diagnostic.

3.10 Fluid Intake / Output — unit: mL or plain cups; cadence: as clinician asks; med-start markers: no; milestones: no; gap tolerance: low if tracking ordered; flag: record only when a clinician asked the family to track; no interpretation.

3.11 Dialysis Session Log — fields: date, attended/missed, duration, how they felt after (self-report); cadence: per session (typically 3x/week); med-start markers: no; milestones: no; gap tolerance: low; flag: attendance and self-report only, no clinical reading.

3.12 Chemo / Infusion Cycle Log — fields: cycle number, date, side-effect notes as observations; cadence: per cycle; med-start markers: YES (treatment start

marker); milestones: mixed (cycle completions); gap tolerance: cycle-based; flag: side effects recorded as observations; no management advice.

3.13 Fall Log — fields: date, circumstances, outcome, who was notified; cadence: per event; med-start markers: no; milestones: no (event log); gap tolerance: n/a; flag: record the event and notifications (ties to Standing Instructions and 483.10(g)(14)); no cause assessment.

3.14 Pediatric Growth Basics — unit: height/length, weight, head circumference (as recorded); cadence: per well visit or as clinician records; med-start markers: optional; milestones: developmental milestones as recorded; gap tolerance: high; flag: record clinician-measured

values; no percentile interpretation by family.

3.15 Temperature — unit: °F/°C; cadence: as needed; med-start markers: optional; milestones: no; gap tolerance: high; flag: record reading only; no fever-management advice.

3.16 Contenance / Toileting (record-only) — unit: plain counts/notes; cadence: daily as relevant; med-start markers: no; milestones: no; gap tolerance: high; flag: observational; no advice.

TEMPLATE TYPE 4: STANDING INSTRUCTION STARTERS

Each: plain-language editable starter wording (no em dashes); rights-vs-request status with citation; acknowledgment to request.

4.1 Notify us before and after falls or injuries — Wording: "Please call [name] at [number] right away if [person] has a fall or any injury, and tell us what happened and what was done." Status: RIGHT. 42 CFR 483.10(g)(14) requires the facility to notify the representative or interested family of an accident with injury that has potential for requiring physician intervention, and of significant status changes. Acknowledgment: ask for the notification noted in the care plan and confirmed in writing.

4.2 Notify us of medication changes — Wording: "Please tell [name] before or when any medication is started, stopped, or changed." Status: RIGHT for significant treatment changes (42 CFR 483.10(g)(14)(C)); routine minor changes may be a

REQUEST. Acknowledgment: note in the care plan; ask who will call.

4.3 Notify us before a room or roommate change — Wording: "Please tell [name] before any room or roommate change."

Status: MOSTLY REQUEST, with partial backing. Facilities must give notice of room changes and residents have rights around transfers within a facility (42 CFR 483.10(e)(6)-(7), 483.15); a roommate-preference notification is largely a request. Acknowledgment: get it added to the care plan.

4.4 Notify us before treatment changes — Wording: "Please tell [name] before starting or stopping any treatment or therapy." Status: RIGHT for significant changes (483.10(g)(14)(C)); REQUEST for

minor ones. Acknowledgment: care plan entry.

4.5 Get our okay before outside billable services — Wording: "Please get [name]'s okay before sending [person] to any outside dentist, podiatrist, lab, or other service that may be billed to us." Status: REQUEST (with related protections). Not a blanket federal right, but the No Surprises Act requires good-faith estimates for uninsured/self-pay (45 CFR 149.610) and notice-and-consent for certain out-of-network non-emergency services.

Acknowledgment: written note in the file; ask for advance cost estimates.

4.6 Include us in care plan meetings — Wording: "Please include [name] in all care plan meetings and tell us the schedule in advance." Status: RIGHT. 42 CFR 483.21

and 483.10 give residents and representatives the right to participate in care planning. Acknowledgment: get the meeting schedule; confirm the family is on the notification list.

4.7 No discharge talk without us present

— Wording: "Please do not discuss discharge or transfer decisions without [name] present or informed." Status: PARTIAL RIGHT. The family has a right to notice of a transfer/discharge decision (483.10(g)(14)(D), 483.15) and to appeal; being "present for every discussion" is partly a request. Acknowledgment: care plan note; confirm the notice process.

4.8 Dietary restrictions on file —

Wording: "Please keep these dietary restrictions on file: [list]. Tell us if they cannot be followed." Status: REQUEST

backed by care-planning duties (483.60 dietary; 483.21 care plan must reflect needs/preferences). Acknowledgment: confirm it is in the care plan and dietary system.

4.9 Visitor and information-sharing preferences — Wording: "Please share information with these people: [list]. Please follow these visitor preferences: [list]." Status: MIXED. Residents have visitation rights (483.10(f)(4)) and control over who gets information (privacy/HIPAA and 483.10). Acknowledgment: confirm the authorized-contact list on file.

4.10 End-of-life / DNR communication protocol — Wording: "Our end-of-life wishes are in the POLST/DNR dated [date], kept at [location]. Please call [name] before any change and follow the signed

orders." Status: RIGHT to have advance directives honored (Patient Self-Determination Act; 42 CFR 489 subpart I; 483.10(c)(6) right to formulate directives). A POLST/DNR is a physician-signed order EMS/staff must follow. Acknowledgment: confirm the order is in the chart and flagged; verify EMS-followable form.

4.11 Notify us of any hospital transfer immediately — Wording: "If [person] is sent to the hospital, please call [name] right away and tell us where and why."

Status: RIGHT (483.10(g)(14)(D) transfer notice; bed-hold notice under 483.15).

Acknowledgment: care plan note; confirm the after-hours contact path.

GLOSSARY (words families hear on the phone; for label wording)

- **MDS (Minimum Data Set):** the standardized resident assessment nursing homes complete; families hear "MDS coordinator."
- **Care conference / care plan meeting:** the scheduled meeting to review the resident's plan; families have the right to attend.
- **Level of care (LOC):** the assessed intensity of care needed; drives Medicaid waiver and facility eligibility.
- **PASRR (Preadmission Screening and Resident Review):** screening for mental illness/intellectual disability before nursing home admission.

- **PRN:** "as needed" (given only when needed).
- **Discharge summary:** the document summarizing a stay and next steps.
- **EOB (Explanation of Benefits):** insurer statement of what was billed, paid, and owed; not a bill.
- **Prior authorization (prior auth):** insurer approval required before a service is covered.
- **Appeal levels:** the ordered steps to challenge a denial (e.g., BFCC-QIO, then QIC reconsideration for Medicare).
- **BFCC-QIO:** Beneficiary and Family Centered Care Quality Improvement Organization; handles fast discharge appeals (Acentra in 29 states; Commence, formerly Livanta, in the rest).

- **Important Message from Medicare (IM):** hospital notice of discharge appeal rights, given within 2 days of admission.
- **Notice of Medicare Non-Coverage (NOMNC):** notice that SNF/home health/hospice coverage is ending, with appeal instructions.
- **MOON (Medicare Outpatient Observation Notice):** notice that a hospital patient is in observation, not admitted.
- **Bed-hold:** policy on holding a facility bed during a hospital stay.
- **Spend-down:** reducing countable assets/income to qualify for Medicaid.
- **Lookback:** the period (commonly 5 years) Medicaid reviews for asset transfers.

- **HCBS waiver:** Home and Community-Based Services Medicaid waiver for care outside a nursing home.
- **POLST/MOLST:** physician/medical orders for life-sustaining treatment; EMS-followable, for the seriously ill; distinct from a living will (a planning document).
- **Healthcare proxy / healthcare POA:** the person named to make medical decisions; the two terms mean the same function, named differently by state.
- **Ombudsman:** the long-term care advocate who helps resolve facility complaints.
- **Involuntary discharge:** facility-initiated discharge/eviction; allowed only for six federal reasons.

EMERGENCY CARD FIELD SPEC

(verified vs File of Life / EMS)

Fields: full name; date of birth; primary language; emergency contacts (name, relation, phone) and the designated point person/decision-maker; allergies; blood type; current medications with dosages; major health conditions; recent surgeries; primary physician (name, phone); insurance/Medicare/Medicaid info; DNR status (yes/no + where the order lives); POLST/MOLST (yes/no + location); healthcare proxy/POA (name, phone + where the document lives); advance directive location; mobility/communication/dementia notes; religion/faith preference (optional); where the original documents are kept. Note the ICE distinction: the Emergency Card is fuller than an ICE contact card.

Recommendations

Build order (staged, population-driven):

1. **Phase 1 (largest populations, highest coordination pain):** Skilled Nursing Facility/Nursing Home (1.1), Hospital Stay (1.4), Home Care with Family Caregiving (1.5), Home Dementia Caregiving (1.14). Pair with Projects: Hospital Discharge Planning (2.5), Medical Records Request (2.6), Medicare Denial/Discharge Appeal (2.3), POA/Advance Directive Setup (2.11). Progress: weight, BP, glucose, fall log, mood/behavior. Standing Instructions: 4.1, 4.2, 4.6, 4.10. Rationale: dementia is the largest single condition category (26% of caregivers) and nursing

home/hospital/home settings carry the largest raw caregiver counts.

2. **Phase 2: Short-Term Rehab (1.2),** Assisted Living (1.7), Memory Care (1.8), Home Health (1.6), Post-Stroke (1.13). Projects: Medicaid LTC (2.1), HCBS Waiver (2.2), Ombudsman Complaint (2.7), FMLA (2.12), Involuntary Discharge Appeal (2.14), Spend-Down (2.15). Progress: mobility/rehab, wound, pain, sleep, appetite.
3. **Phase 3 (smaller but high-burden):** IRF (1.3), Hospice (1.9), Dialysis (1.10), Cancer/Infusion (1.11), Pediatric Serious Illness (1.12). Projects: SSDI/SSI (2.8), VA (2.9), Guardianship (2.10), Transfer (2.13), Prior-Auth Appeal (2.4), EOB Reconciliation (2.16). Progress:

dialysis log, chemo log, pediatric growth, fluid I/O.

Thresholds that change the plan: If usage analytics show a setting is opened far more than its phase implies (e.g., assisted living), promote it. If a template's checklist items are frequently edited/deleted by users, shorten it. If translation review finds a role label untranslatable, switch to a function-descriptive label.

Design mandates:

- Every template ships ~6 contact roles, 4-5 threads, ~10 checklist items, a handful of document slots. Keep strings short and translatable.
- Default Spanish to formal *usted*; address elders as Señor/Señora.
- For "hospice," "power of attorney," "advance directive," and "social

worker," never use bare cognates; use a descriptive phrase plus a plain gloss, reviewed by native speakers.

- Support multiple decision-makers and a designated family point person; add a "who decides / who receives information" setting.
- Ship a global rule that no string may instruct, remind, interpret, or advise; every Progress preset and checklist item is record-only.

Open Questions / Judgment Calls for Coding

1. **Role-label localization vs literal translation:** decide per role whether to translate the US title or substitute a function-descriptive label (strongly recommended for "social worker" and "case manager" in

Chinese/Arabic). Requires native-speaker validation.

2. **Single vs multiple point persons:** the data supports multiple decision-makers; decide whether the data model allows co-equal point persons or a primary-plus-backup structure, and how that maps to the Emergency Card and Standing Instructions.
3. **Rights-label rendering:** decide the UI convention for distinguishing a federal right from a request (e.g., a small "backed by federal rule" tag vs a "your request" tag) without giving legal advice.
4. **State-variance posture:** decide the exact standing disclaimer wording for high-variance templates (Medicaid, guardianship, assisted living, POA) and whether to prompt

the user to enter their state to adjust generic strings.

5. **Volatile variables:** BFCC-QIO contractor names/regions, SSI resource limits, and civil-money-penalty ranges change; store these as updatable config, not hard-coded strings, or omit them from shipped templates entirely.
6. **Progress advice-boundary enforcement:** decide how to technically prevent free-text Progress notes from drifting into self-diagnosis (e.g., neutral field labels, no "normal range" hints, no color-coding of values).
7. **Hospice/end-of-life sensitivity:** decide whether to soften death-related copy for taboo-sensitive languages (Chinese number-4 and

"death" avoidance) and how to gloss "hospice" so comprehension is preserved without stigma.

8. **Checklist depth vs shallowness:** the ~10-item target may be too long for some settings (home care) and too short for others (Medicaid). Decide whether counts flex per template or stay fixed.

Caveats

- **State variance is large** for Medicaid (LTC, HCBS, spend-down), guardianship, assisted living regulation, POA/advance-directive forms, and involuntary-discharge hearings. These templates must carry a visible "your state may differ" posture and stay generic.

- **Rights vs requests:** the app must not overstate entitlements. Notification of injuries/significant changes, care-plan participation, records access, and involuntary-discharge protections are genuine federal rights (42 CFR Part 483; HIPAA 45 CFR 164.524). Roommate-change notice, presence at every discharge discussion, and consent before every outside service are largely requests with partial backing; label them accordingly.
- **Some numbers evolve:** SSI resource limits, civil money penalty ranges, and BFCC-QIO contractor names (Acentra/Commence) change over time and by region; treat them as variables, not hard-coded strings.

The Livanta-to-Commence rename took effect August 18, 2025.

- **Cultural findings are directional, not deterministic:** a Shanghai study found four decision-making profiles within one population, so the app should ask, never assume, and must avoid stereotyping. Some cultural sources are expert commentary rather than peer-reviewed studies; the decision-making literature (Blackhall et al. JAMA 1995; Yang et al. IJERPH 2022) is the stronger backbone.
- **No dedicated comprehension study** was found for Arabic "hospice" among US Arab Americans, or for "healthcare proxy" and "case manager" across all three target languages; those label choices should

be validated with native-speaker testing before release.

- **CMS checklist:** Health Trail mirrors the structure of "Your Discharge Planning Checklist" and other official checklists but must write its own strings and remove all clinical instruction to stay record-only.
- **The caregiving statistics** are from the 2020 NAC/AARP report (data collected 2019); a 2025 update exists and should be checked for revised counts before finalizing build-order priorities.